

1. Administrative & Study Identification

Full Study Title: A Phase 1 Open-label Study to Investigate PF-08046876 in Adult Participants With Advanced Solid Tumors
Short Title/Acronym: A Study to Learn About the Study Medicine Called PF-08046876 in People With Advanced Solid Tumors
Protocol Number: C5951001 **NCT Number:** NCT07090499 **Sponsor Name:** Pfizer
Investigational Product: PF-08046876 (Integrin Beta-6 (IB6) Directed Top1 Inhibitor / Antibody Drug Conjugate)
Phase of Development: Phase I **Medical Monitor:** Pfizer Clinical Medical Director

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and identify the Recommended Phase 2 Dose (RP2D) of the study drug (PF-08046876) in participants with advanced solid tumors. **Secondary Objectives:** To evaluate single and multiple dose pharmacokinetics (PK), immunogenicity, Progression-Free Survival (PFS), and Overall Survival (OS).

2.2 Background & Rationale PF-08046876 is an investigational anticancer therapy called an antibody drug conjugate (ADC) designed to target and kill cancer cells. This study is being conducted to evaluate the safety and preliminary efficacy of this drug in patients with advanced cancers of the bladder, lung, head and neck, esophagus, or pancreas who have progressed or relapsed following standard treatments.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose Escalation and Dose Expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: Phase 1 sizing (approx. 50-100 participants across cohorts) **Number of Sites:** Multiple global sites (USA, CAN) **Estimated Study Start:** Q4 2024 / Q1 2025 **Estimated Primary Completion:** Up to 3 years of long-term follow-up

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. 18 years of age or older. 2. Advanced cancer of the bladder, lung, head and neck, esophagus, or

pancreas with measurable disease. 3. ECOG Performance status 0-1. Part 1: progression or relapse following standard treatments. Part 2: maximum of 2 prior lines of systemic therapy in the advanced setting.

4.2 Exclusion Criteria 1. Received prior treatment with an antibody drug conjugate with a camptothecin-class payload (e.g., sacituzumab govitecan, trastuzumab deruxtecan). 2. Active anorexia, nausea or vomiting, and/or signs of intestinal obstruction meeting protocol exclusion. 3. Pulmonary disease meeting protocol exclusion.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Multiple dosing levels depending on cohort assignment (Part 1 Dose Escalation), followed by 1 selected dosing regimen (Part 2 Dose Expansion). **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive oncology care. **Prohibited:** Other systemic anti-cancer therapies, particularly concurrent ADCs with a camptothecin-class payload.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	2-4 weeks prior	Informed Consent, Medical History, Tumor Assessments, Baseline Labs
Baseline	Day 0	First IV Infusion, PK/PD Sampling, Safety Audit
Interim	Per Cycle	Safety Labs, PK assessments, Tumor Evaluations (RECIST v1.1)
End of Study	+30 Days	30 days post last study drug administration safety follow-up
Follow-up	Up to 3 years	Long-term follow-up for OS and PFS

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Determination of Recommended Phase 2 Dose (RP2D) based on cumulative safety (Dose-Limiting Toxicities), preliminary anti-tumor activity, and Pharmacokinetics. **Measurement Method:** Clinical safety assessments and RECIST v1.1 evaluation from Baseline to 30 days post last study drug administration.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) defined as the time until death due to any cause. * Progression-Free Survival (PFS). * Pharmacokinetics (PK) of single and multiple doses. * Immunogenicity of PF-08046876.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of AEs and Dose-Limiting Toxicities (DLTs) via investigator assessment and patient report. **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline per GCP guidelines. **Data Monitoring Committee (DMC):** Internal Safety Review Committee (SRC) evaluates cumulative safety data to guide dose escalation decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants who receive at least one dose of the investigational product). Efficacy Evaluable Set for tumor response. **Sample Size Justification:** Determined by standard adaptive Phase 1 dose-escalation rules. **Handling of Missing Data:** Standard oncology methods (e.g., right-censoring for time-to-event endpoints like OS and PFS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring to ensure protocol adherence, subject safety, and data integrity. **Audit Trail:** E-clinical data capture (eCRF) locking and standard data clarification workflows.

11. Study Identifiers & Governance

Primary ID: C5951001 **Registry Number:** NCT07090499 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study to Learn About the Study Medicine Called PF-08046876 in People With Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 Open-label Study to Investigate PF-08046876 in Adult Participants With Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Solid Tumors (Bladder, Lung, Head and Neck, Esophagus, Pancreas) **Diagnosis Threshold:** Histologically/cytologically confirmed advanced disease with measurable lesions. **Medical Methodology:** Integrin Beta-6 Directed Topoisomerase-1 Inhibitor (Antibody Drug Conjugate) **Optimization Target:** Recommended Phase 2 Dose (RP2D) establishment

13. Study Procedures & Methodology

Management Pathway: Part 1 (Dose Escalation) followed by Part 2 (Dose Expansion) **Education Protocol (HCP):** Site initiation visits covering IV administration protocols and strict ADC safety/DLT monitoring. **Education Protocol (Patient):** Thorough informed consent regarding Phase 1 trial expectations and AE self-reporting. **Quality Audit Mechanism:** Safety Review Committee (SRC) cadence for determining subsequent dose levels.

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years for long-term survival follow-up
Onsite Visit Cadence: Frequent during Cycle 1 for PK and DLT tracking, followed by cycle-based administration (e.g., Q3W).
Remote Monitoring Frequency: Intermittent survival status checks post-progression. **Checklist Review Interval:** Routine safety lab reviews prior to each dose administration.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				